



WNHS PROCEDURE	
Pre-conception Consultation for Women with Severe Mental Illness	
Scope (Staff):	Mental health staff working in WNHS
Scope (Area):	Department of Psychological Medicine

1. Aim

The purpose of this guideline is to outline the process for staff offering pre-conception counselling to women with severe mental illness (SMI). These appointments aim to provide:

- Individualised assessment of the women's possible course of mental illness during perinatal period.
- Discussion of the risks and benefits associated with pharmacological and other treatment options.
- Feedback to the woman, and/or her partner/family member/support person, and her treating doctor regarding specific risks during the perinatal period and potential interventions (pharmacological and psychosocial) to strengthen resilience and reduce risk.

The Department of Psychological Medicine aspires to ensure the rights of a woman, her fetus/infant and partner/family member/support person are met according to [Charter of Mental Health Care Principles \(Mental Health Act 2014\)](#).

2. Risk

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Mandatory Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Impact financial operations	☐
Damage NMHS reputation	☐	Impact on Staff Safety	☐
Other	☐		

3. Background

Preconception counselling is a prevention strategy and involves providing health information to women of childbearing age who suffer from a specific chronic condition, with the intention of promoting optimal health and informed choice around pregnancyⁱ.

Preconception counselling has been shown to impact on health behaviours and is now recommended for a number of chronic disordersⁱⁱ. The perinatal period is a time of high risk for women with SMI, with increased risk of relapse, particularly in women who stop taking maintenance medication prior to or during pregnancyⁱⁱⁱ. Women with SMI have increased risk of pregnancy complications, as well as the potential for congenital abnormalities and adverse neurodevelopmental outcomes for the baby, independent of any risk associated with psychotropic medication^{iv, v}. There are implications for the woman's functioning and capacity to parent if she remains or becomes unwell.

4. Referral

- Women must have a diagnosed SMI and be referred by their community mental health practitioner, private psychiatrist or general practitioner. SMI includes:
 - a. bipolar affective disorder type 1
 - b. schizophrenia
 - c. past history of puerperal psychosis around pregnancy and childbirth
 - d. women with other mental health issues such as eating disorder or obsessive compulsive disorder or co-morbid severe personality disorder may also be eligible for this service.
- The Department of Psychological Medicine Outpatient Triage Officers review the referral and may contact the referring practitioner for additional information on past psychiatric history and treatment.
- An initial appointment for pre-conception assessment and counselling is offered.
- The patient is encouraged to attend with her partner/support person. Refer to [Rights of Carers and Other Support Persons](#).

5. Assessment

Women with SMI have an increased risk of co-morbid physical illness, drug and alcohol disorders, other mental illness, psychological adversity and previous trauma. Given the complexity of factors involved a thorough bio-psycho-social assessment is required, with particular consideration of:

- Aspects of mental illness:

- Clinical features of the woman's illness including relapse frequency and severity, and clinical presentations including past hospitalisation and any previous perinatal episodes
- Risk assessment including previous suicide/self-harm attempts; previous aggression or risk of harm to others including children; history of neglect of care for self and/or children including attendance for health and mental health related treatment
- Level of insight/capacity to be involved in collaborative planning/awareness of symptoms of relapse
- Current and previous treatments, efficacy and compliance.
- Past medical history (identifying medical co-morbidities likely to impact e.g. obesity, type 2 diabetes), obstetric/gynaecological history, drug and alcohol (including smoking) history and current use, and diet and nutrition.
- Family history including perinatal relapses of family members.
- Personal history including patient's own developmental experience and attachment relationships and experience of previous trauma.
- Current levels of perceived practical, emotional and social support; nature and stability of living situation; presence of family and domestic violence.
- Hopes, fears and experience around pregnancy and parenting, and potential risks of mental illness in child.

Key Points for Discussion with Patient

It is anticipated that the following issues will be addressed and discussed with the women and her partner/carer.

- Potential risk of relapse of mental illness during the perinatal period including effects of mental illness on pregnancy and fetus
- The possible risk of offspring inheriting the same or similar illness
- Discussion of risk/benefit of current medications during perinatal period and discussion on other medication options (See section 9 [Women's and Perinatal Mental Health Referral and Management guideline](#))
- Discussion of non-pharmacological interventions including accessing social support, psycho-education and psychological therapies as indicated
- Encourage, recommend and refer as appropriate, to intervention for drug and alcohol issues or physical health co-morbidity
- Discussion re healthy lifestyle, diet, sensible regular exercise
- Recommend early referral to Childbirth and Mental Illness antenatal clinic or early review by psychiatrist with liaison between obstetrician and GP if a pregnancy is confirmed, so that a comprehensive plan for obstetric, paediatric and psychiatric monitoring can be put in place^{vi}.
- Patients should be provided with endorsed written information ([BILLS](#)) (see [MP 0069/17](#) Mental Health Consumer Information Policy below and website).

Clinicians may consider supplementing with more specific written information relevant to the perinatal period (see “Useful resources”).

Discussion of the risks and benefits of the use of any psychotropic medication in pregnancy should be clearly documented in the patient’s medical record, highlighting the specific issues covered. A letter summarizing the assessment, discussion and any recommendations is forwarded to key stakeholders who may include treating psychiatrist and/or community mental health team, general practitioner and obstetrician. A copy is provided to the woman (and her partner/carer if the woman consents).

Appropriate written information should be provided to the patient and partner/support person/family member and documented.

[Department of Health Choice and Medication Printable Leaflets for Psychotropic Medications Website](#)

Related internal policies, procedures and guidelines (if required)

[Women’s and Perinatal Mental Health Referral and Management Guideline –link Mental Health Act 2014](#)

[Carer’s Recognition Act 2004](#)

[Charter of Mental Health Care Principles](#)

[Chief Psychiatrist’s Guidelines 2015](#)

[WNHS Discharge policy](#)

Useful resources

<http://www.maprc.org.au/nramp>

<http://mothertobaby.org/fact-sheets>

www.uktis.org

[Centre for Women’s Mental Health](#)

Pre-Conception Consultation for Women with Severe Mental Illness

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NSQHS Standards Applicable:	☐  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management		☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
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The health impact upon Aboriginal people has been considered, and where relevant incorporated and appropriately addressed in the development of this policy (insert ISD Number). (Please refer to the Aboriginal Health Impact Statement and Declaration for Department of Health and Health Service Provider Guidelines – please delete once you have completed this).

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ⁱ de Weerd, S., et al., *Preconception care: a screening tool for health assessment and risk detection*. Prev Med, 2002. 34(5): p. 505-11.

ⁱⁱ Winterbottom JB, Smyth RM, Jacoby A, Baker GA. *Preconception counselling for women with epilepsy to reduce adverse pregnancy outcome*. Cochrane Database Systematic Review. 2008 Jul 16;(3):CD006645. doi: 10.1002/14651858. CD006645.pub2.

ⁱⁱⁱ Viguera, A.C., et al., *Risk of recurrence in women with bipolar disorder during pregnancy: prospective study of mood stabilizer discontinuation*. American Journal of Psychiatry, 2007. 164(12): p. 1817-24; quiz 1923

^{iv} Jablensky A., et al., *Pregnancy, delivery, and neonatal complications in a population cohort of women with schizophrenia and major affective disorders*. Am J Psychiatry, 2005. 162: 79-91.

^v Boden, R., Lundgren M., Brandt L., Reutfors J., H., *Antipsychotics during pregnancy: relation to fetal and maternal metabolic effects*. Archives of General Psychiatry, 2012. 69: 715-721.

^{vi} Galbally, M., et al., *Management of antipsychotic and mood stabilizer medication in pregnancy: recommendations for antenatal care*. Australian and New Zealand Journal of Psychiatry, 2010. 44(2): p. 99-108.